

DISCOVERY REPORT

Eligibility Assessment Flow

Source-to-destination implementation plan and approval gate

Project: Rejuvonix telehealth website

Source: rejuvonix.com/eligibility

Destination: rejuvonix.otrayoe.chatgpt.site

Prepared: August 25, 2026

Status: Plan only — implementation blocked pending approval

Decision gate

Do not implement the full source intake yet. It gathers health history, prescription-use details, identity/contact data, uploads, consent, and shipping information, while the destination currently has no suitable intake backend or protected-data controls. Approval is required for either a non-submitting, non-PHI pre-screen or a full intake backed by reviewed clinical, privacy, security, and operational infrastructure.

Executive outcome

The source flow can be reproduced technically, but not safely as a complete production intake using the destination's current architecture. The destination has a three-question informational questionnaire and no intake API, protected storage, authenticated member workflow, upload service, consent ledger, or established health-data analytics policy.

1. Recommended near-term option: build a non-submitting, non-PHI eligibility pre-screen that routes qualified visitors to a clearly disclosed next step.
2. Alternative: pause implementation until the prescribing medical group, privacy counsel, security owner, and operations team approve the full data lifecycle and clinical logic.
3. Do not silently copy promotional claims, testimonials, approval language, or medical outcome logic without review.

1. Scope, methods, and system context

This report documents the visitor-facing source assessment, identifies assessment-related calls to action across both sites, maps the source flow to the current destination architecture, and defines the minimum decisions required before code changes. The assessment was inspected without bypassing authentication and without using real patient information.

Sites reviewed

Surface	Role	Observed access
rejuvonix.com	Approved public source website	Public pages and eligibility flow reviewed
rejuvonix.com/eligibility	Approved source assessment	Visitor-facing flow and branching documented
rejuvonix.otrayoe.chatgpt.site	Destination website	External URL returned HTTP 401 during review; local project inspected
Local Rejuvonix repository	Destination implementation source	Routing, components, storage, tests, and hosting configuration inspected

Destination architecture

4. Next.js 16 / React 19 application using the Vinext compatibility layer and Cloudflare-oriented deployment configuration.
5. Public routes are implemented in the app directory; shared public chrome is reused by many pages, but some page-level UI remains duplicated.
6. The only observed API surface is a health endpoint. There is no patient-intake submission API.
7. The current /get-started experience contains three informational questions and does not persist or submit answers.
8. The sign-in route is a visual mockup, not a completed authenticated member portal.
9. The hosting configuration exposes no bound D1 database or R2 object storage, and the D1 schema is empty.
10. No established analytics event system or destination-specific privacy/consent capture mechanism was found for assessment answers.

Public source routes crawled

/, /about, /contact, /disclaimer, /eligibility, /hipaa, /privacy, /terms, /waitlist, /treatments/glp-1-microdose, /treatments/semaglutide, /treatments/tirzepatide

Primary sources: [Source home](#) | [Source eligibility](#) | [Destination](#)

2. Source eligibility flow map

The source is a multi-stage intake with 44 configured fields, conditional questions, early lead transmission, automated disqualification rules, and a post-submission approval/checkout path. The following map separates general wellness screening from health, prescription, contact, consent, and fulfillment data.

High-level sequence

11. Start eligibility → wellness and motivation questions
12. Sex/pregnancy branch → body measurements and BMI gate
13. Name/contact capture → early lead server call and Meta matching
14. Medical history → surgery, GLP-1, medication, and allergy branches
15. Risk-tolerance and promotional interstitial
16. Date of birth and shipping address
17. Accuracy/terms/privacy/telehealth and automated-contact consents
18. Client disqualifier check → server submission → curating state
19. Approved result → pharmacy promotion → treatment checkout

Important sequencing risk

The source makes an early lead server call, including name, email, and phone and Meta matching, as soon as those fields validate—before the visitor reaches the final consent screen. That behavior should not be copied without explicit privacy and marketing-consent approval.

Stage A — General eligibility

Step	Prompt / field	Options or validation	Classification
1	Primary motivation	Long term health; Feel better day to day; Confidence and appearance; Reduce health risks	General screening
2	Current activity	Very active; Moderately active; Not very active	Lifestyle
3	Typical sleep	>9 hours; 7–9; <7; varies/trouble	Lifestyle / health-adjacent
4	Stress frequency	Rarely; Moderately; Often	Health-adjacent
5	Gender	Male; Female	Sensitive personal data
6	Pregnancy branch	If female: No / Yes. Yes disqualifies.	Sensitive health data
7	Height, current weight, goal weight	Height 3–8 ft and 0–11 in; weights 50–600	Health data
8	BMI interstitial	BMI below 25 disqualifies; promotional outlook/timeline copy shown	Derived health data / claim
9	First and last name	Both required	Identity data
10	Email and phone	Both required and validated	Contact data

Stage B — Medical history and treatment context

Prompt / field	Choices / conditional behavior	Sensitivity
Body-shape selection	Four visual choices	Body/health profile
Ethnicity	Six choices	Sensitive demographic data
Current health effects	Multi-select: none, low libido, skin, brain fog	Health symptoms
Diagnoses / contraindications	Multi-select list covering thyroid cancer history, MEN2, cancer, substance misuse, eating disorder, serious mental-health conditions, pancreatitis, diabetes, liver/kidney/gallbladder disease, and persistent hypoglycemia	High-sensitivity medical history
Prior surgeries	Yes/No; details required when Yes	Medical history
Weight-loss surgery	None; bypass; duodenal switch; lap band; sleeve	Medical history
GLP-1 history	Never taken / Taken	Prescription treatment history
If GLP-1 taken	Medication name, last injection dose, last-dose date, duration, optional side effects, photo availability/upload, data visibility confirmation, optional preferred starting dose, required dose acknowledgment	Prescription and upload data
Medications / supplements	Yes/No; details required when Yes	Medication data
Specific allergies	None; glycine; B12; B3; unsure	Medical data
Other allergies	Yes/No; details required when Yes	Medical data
Resting heart rate	Unknown; <70; 70–85; >85	Health measurement
Tolerance for side-effect disruption	High; Moderate; Low	Treatment preference

A promotional testimonial interstitial claiming a 60% outcome appears after the risk-tolerance question.

Automated medical disqualifiers observed

20. Medullary thyroid carcinoma (personal history)
21. Family history of medullary thyroid carcinoma
22. Multiple Endocrine Neoplasia type 2 (MEN2)
23. Pancreatitis
24. Active gallbladder disease

Other serious diagnoses appear in the questionnaire but were not observed in the same client-side automated-disqualifier list. That discrepancy requires clinical review; the destination should not infer or expand medical decision logic.

Stage C — Completion, consent, and outcomes

Area	Observed behavior	Implementation concern
Date of birth	Required; accepted age range 18–100	Identity and health eligibility; verify age rules and state/service restrictions
Shipping address	Line 1 required; line 2 optional; city, state, and ZIP required	Fulfillment data should not be collected without a defined operational need and protected lifecycle
Accuracy / legal consent	Required acknowledgment referencing Terms, Privacy, and Telehealth	Exact approved legal text and versioned consent evidence are needed
Automated contact consent	Required	Marketing/communications counsel should review channel, scope, revocation, and recordkeeping
Submission	Client disqualifier check, then server call, curating state, approved outcome, pharmacy promotion, treatment checkout	Do not imply treatment or prescription is guaranteed; server-side clinical control required
Errors	Retry or review responses	Preserve answers securely and avoid exposing sensitive content in logs or URLs
Exit	Header logo exits without confirmation	Risk of accidental abandonment
Refresh / return	Answers reset; no complete persistence observed	Clarify expected retention and restart behavior
Back navigation	Visitor can return and change a disqualifying answer	Server must independently validate; client-side outcomes are not authoritative

Automated outcome rules observed

- 25. BMI below 25 → ineligible
- 26. Pregnant, breastfeeding, or trying to become pregnant → ineligible
- 27. Personal or family history of medullary thyroid carcinoma → ineligible
- 28. MEN2 → ineligible
- 29. Pancreatitis → ineligible
- 30. Active gallbladder disease → ineligible

Outcome-language concern

The source displays “You’re approved” before checkout. A destination implementation should distinguish questionnaire screening from an independent prescriber’s medical decision and should not promise treatment or a prescription.

3. Source CTA inventory

The source repeatedly directs visitors toward eligibility using global navigation and page-specific actions. Informational links were kept separate from patient-intake actions.

Location	CTA text / element	Observed intent
Global site chrome	Build My Protocol	Open or navigate to eligibility/intake
Global site chrome	Build My Protocol →	Open or navigate to eligibility/intake
Global site chrome	Check Eligibility	Navigate to eligibility
Homepage hero/content	Get Started	Start patient-intake journey
Homepage cards	Hair Restoration; Skin Regeneration; Performance; Sexual Health	Card actions appear intake-oriented and require product/path review
About page	Schedule Your Consultation	Patient-intake / consultation action
Treatment pages	Eligibility- or protocol-oriented actions	Start assessment or proceed toward treatment

Informational navigation

Learn More, About, Contact, treatment-detail navigation, Terms, Privacy, HIPAA, disclaimer, and other policy links should remain informational unless their actual purpose clearly starts assessment. The destination should not convert general discovery links into eligibility actions merely to increase CTA consistency.

Source behavior notes

31. The source mixes direct navigation, modal-like progression, promotional interstitials, medical questions, and checkout-oriented steps in one journey.
32. CTA language varies across pages, including Build My Protocol, Get Started, Check Eligibility, and Schedule Your Consultation.
33. Some cards use treatment-category labels that do not by themselves make the destination or next action clear.

4. Source-to-destination screen mapping

Source stage	Destination equivalent	Disposition
Eligibility start / wellness questions	New native /eligibility route using existing typography, controls, and responsive patterns	Suitable for an approved non-PHI pre-screen
Sex, pregnancy, BMI	New accessible conditional form steps	Requires clinical review; pregnancy and BMI are health screening
Name, email, phone	No safe existing intake equivalent	Do not implement early lead submission without consent and privacy approval
Medical history / contraindications	No existing protected medical-intake system	Blocked for full implementation
GLP-1 history and photo upload	No secure upload or object-store binding	Blocked; do not store in public client or generic logs
DOB and shipping address	No fulfillment backend	Blocked unless approved operational requirement and protected lifecycle exist
Legal / telehealth / contact consent	Destination has policy links but no versioned consent ledger	Needs counsel-approved language and evidence model
Automated disqualification	Could be reproduced in client UI, but client logic is not authoritative	Needs server validation and clinical ownership
Approved result and checkout	No equivalent prescribing or checkout workflow identified	Do not copy; define reviewed next step
/get-started	Existing three-question informational questionnaire	Preserve initially; later redirect only after approved route replaces it

Proposed route

Use /eligibility as the canonical destination route. Preserve /get-started during implementation and testing; once the new flow is approved, either convert it to a compatibility redirect or present a deliberate handoff so existing links do not break.

Presentation approach

34. Build the flow natively—do not iframe the source.
35. Reuse destination form controls, buttons, spacing, typography, color tokens, focus treatment, and responsive containers.
36. Use a clear step title, visible progress, back/continue controls, inline validation, and a restart/exit pattern with confirmation when answers would be lost.
37. Preserve answers when moving backward, subject to the approved privacy model.

38. Keep mobile controls reachable without horizontal overflow and announce step and validation changes to assistive technology.

5. Destination CTAs

Assessment-related CTAs to connect after approval

Destination area	Likely component / file	Connection rule
Homepage eligibility actions and modal result	app/page.tsx	Route intentional assessment actions to /eligibility; preserve modal accessibility work and eligibility logic unless separately approved
Shared header and footer	Shared SiteChrome component	Connect only Get Started / Check Eligibility / assessment-oriented labels; leave informational navigation unchanged
Shared product/treatment CTA	ProductPage component	Connect assessment actions while preserving treatment detail navigation
Wegovy experience CTA	WegovyInjectionExperience component	Connect only the assessment-oriented action
Compounded, how-it-works, safety, support, treatments, sign-in	Corresponding app routes/components	Audit each CTA by visible intent; connect assessment actions, retain informational and authentication actions

CTAs that should remain unchanged

39. Learn More and treatment-detail links that lead to explanatory content.
40. Header/footer About, Contact, treatment navigation, policy, privacy, HIPAA, terms, and disclaimer links.
41. Sign-in controls, which should not be redirected to eligibility.
42. Any support or contact link whose visible purpose is assistance rather than assessment.
43. Carousel, BMI calculator, treatment cards, and other unrelated interactions unless a specific control is explicitly assessment-oriented.

Consistency rule

Use one approved assessment label site-wide where practical, but do not change informational labels or clinical/product wording solely for uniformity. Every connected control should land on the first assessment screen, not a mid-flow state.

6. Proposed data-handling approach

Option A — Non-submitting pre-screen (recommended first)

44. Limit questions to the minimum approved eligibility indicators and avoid name, contact, free-text medical history, prescription details, uploads, DOB, and shipping address.
45. Keep answers in component memory only; do not place answers in URLs, analytics, console output, error reports, or persistent browser storage.
46. Provide clear screening language: the result is informational and does not guarantee treatment, prescribing, or fulfillment.
47. Route visitors to a reviewed next step without transmitting health answers.
48. Use synthetic data only in tests and screenshots.

Option B — Full source-equivalent intake

Before implementation, the project needs all of the following:

Capability	Minimum requirement
Clinical governance	Named clinical owner; approved question set; disqualifier and outcome logic; prescriber-independent decision path; emergency/urgent-care limitations
Backend	Authenticated or otherwise protected intake API; strict server validation; idempotent submission; controlled error handling
Storage	Encryption in transit and at rest; defined database records; secure file uploads; least-privilege service bindings
Access control	Role-based access, staff authentication, audit logging, and separation of operational and clinical permissions
Privacy lifecycle	Data minimization; notice; purpose limitation; retention/deletion schedule; access/correction/deletion workflow; vendor inventory
Consent	Counsel-approved telehealth, privacy, terms, and automated-contact language with version, timestamp, source, and revocation handling
Analytics	Explicit policy prohibiting health answers, contact fields, uploads, and derived eligibility outcomes from general analytics/advertising payloads
Operations	Eligible states/services, prescribing medical group, pharmacy/fulfillment handoff, support and escalation procedures
Security	Threat model, log review, secrets management, abuse controls, incident response, and vendor agreements where required

Prohibited client-side handling

49. No sensitive answers in query strings, hashes, page titles, referrers, client logs, or analytics event properties.

- 50. No persistent localStorage/sessionStorage cache for a full medical intake without explicit review.
- 51. No direct upload to a public or unprotected bucket.
- 52. No client-only determination that is represented as clinical approval.
- 53. No early marketing lead transmission before a valid, reviewed consent basis.

7. Expected implementation surface

Exact files should be confirmed immediately before implementation because the repository is active. The expected surface below is deliberately narrow and does not authorize changes.

Area	Expected change	Notes
New eligibility route	app/eligibility/page.tsx and narrowly scoped local components/tests	Native flow and accessible step management
Shared assessment components	Reuse or add within existing component conventions	Avoid a second design system or duplicated chrome
Homepage	app/page.tsx	Connect approved assessment CTAs only; avoid unrelated modal/carousel changes
Shared navigation	SiteChrome component	Connect approved header/footer assessment controls
Product pages	ProductPage and specific experience component	Only assessment-oriented actions
Current questionnaire	app/get-started/page.tsx	Compatibility route or handoff after approval
Tests	Existing contract/UI tests or narrowly scoped additions	Cover route, branching, keyboard, validation, and no sensitive telemetry
Backend/config	Not defined	Stop if full intake is approved until backend/security design is separately authorized

Areas intentionally outside scope

- 54. Unrelated destination page redesigns, typography, navigation structure, treatment content, pricing, product claims, authentication, database, worker, or deployment behavior.
- 55. Source website changes or private/patient systems.
- 56. Generated replacements for real product photography.
- 57. Claims editing without an authoritative source and appropriate owner review.
- 58. Production deployment, domain changes, merge, or publication.

8. Compliance and visitor-understanding review

This is a technical and content-risk review, not legal advice or a compliance certification. Confirmed observations are separated from decisions requiring healthcare counsel, privacy counsel, the prescribing medical group, pharmacy, or marketing review.

Claims and representations requiring review

Observed source representation	Why it needs review	Owner
"Lose weight without cravings—starts this week"	Outcome and timing claim; may imply predictable individual response	Clinical + marketing/legal
"92% success outlook"	Quantified success representation; substantiation and definition of success needed	Clinical + marketing/legal
"~18% body weight" and goal timeline	Quantified efficacy and projected personal timeline	Clinical + marketing/legal
"Down 38 lbs," real-member testimonials, before/after material	Endorsement, typicality, disclosure, and image authenticity concerns	Marketing/legal
"You're approved"	May blur screening, medical eligibility, and independent prescribing decision	Clinical + legal
"Safest, highest-quality," testing every lot, "All dosages"	Comparative/superlative, quality, testing, and availability claims need precise substantiation	Pharmacy + quality + legal
Broad HIPAA representations	Applicability and actual safeguards depend on entities, data flows, vendors, and contracts	Privacy/security counsel

Prescription and compounded-drug boundaries

59. Clearly distinguish FDA-approved branded products from compounded preparations whenever either is discussed.
60. Do not state or imply that compounded drugs are FDA-approved.
61. Do not promise a prescription or imply the prescriber's independent decision is predetermined.
62. Where dosing language appears, use: "Your prescriber will determine your dose and treatment schedule based on your individual medical needs."
63. Do not add efficacy, safety, weight-loss, anti-aging, hormone, sexual-health, hair-restoration, or skin-restoration claims without current authoritative substantiation and documentation.

Authoritative review sources: [FDA: Compounding and FDA Q&A](#) | [FDA: Concerns with unapproved GLP-1 drugs](#) | [FTC: Health Products Compliance Guidance](#) | [FTC: Endorsement Guides Q&A](#)

9. Accessibility and interaction requirements

Any approved implementation should meet WCAG 2.2 Level AA where applicable and preserve the destination's established visual system.

Interaction	Required behavior
Step changes	Move focus to the new question heading/group and announce step/progress without duplicate ARIA output
Keyboard	Logical tab order; all choices and controls operable; no keyboard traps; visible focus
Validation	Programmatic labels, field instructions, clear inline errors, error summary/focus where appropriate
Back/restart/exit	Preserve answers on back; confirm destructive restart/exit; return focus predictably
Conditional fields	Reveal programmatically, announce when needed, and remove hidden required states
Mobile	No horizontal overflow; reachable controls and safe-area spacing at 390px and 320px
Motion	Honor prefers-reduced-motion and avoid unnecessary transitions
Sensitive content	No answer content in accessible names, URLs, logs, or telemetry beyond what the user must hear on-screen
Outcome	Explain that screening does not guarantee treatment; provide a safe next step and a way to review answers

Browser-test status

Visual confirmation remains pending

The external destination returned HTTP 401 during review, and prior browser automation did not have an active browser. Source-level findings are complete, but desktop/mobile screenshots and visual behavior at 1440, 1024, 768, 390, and 320 pixels must be captured during implementation verification.

10. Decisions and approval required

Choose one implementation boundary before any eligibility code is added:

Decision	Scope	Readiness	Recommendation
A. Non-submitting pre-screen	Minimum approved eligibility indicators; no identity/contact, free-text medical history, uploads, DOB, or address	Can proceed after clinical/copy approval and CTA confirmation	Recommended first
B. Full source-equivalent intake	Medical history, prescription details, identity/contact, uploads, consent, address, outcome and handoff	Blocked until backend, security, consent, clinical, state/service, pharmacy, and privacy requirements are approved	Do not begin yet

Approval questions

64. Should the first implementation be Option A, the non-submitting non-PHI pre-screen, or should work pause for Option B infrastructure and governance?
65. Which destination CTA labels and page locations are explicitly approved to start assessment?
66. What is the reviewed next step for visitors who pass or fail the pre-screen?
67. Who owns approval of the medical question set and disqualifier logic?
68. Which states and services are in scope, and how should unavailable services be communicated?
69. What privacy, telehealth, terms, and automated-contact language is approved, and how must consent evidence be retained?
70. Which source promotional interstitials, claims, testimonials, before/after material, pricing, and pharmacy representations—if any—are approved to copy?

Recommended next step

Approve a narrow implementation plan

Authorize a native /eligibility pre-screen that keeps answers in memory, collects no identity or contact information, sends no health answers to analytics or a backend, uses clearly non-clinical outcome language, and connects only the explicitly approved assessment CTAs. Continue to hold the full intake until its protected-data and clinical workflow are designed and reviewed.

Record of intentional exclusions

71. No code was changed as part of the discovery pass.
72. No real patient data was used or submitted.

- 73. No private systems, authenticated patient areas, credentials, environment secrets, or backend data were accessed.
- 74. No production site, domain, deployment, branch, pull request, or source website was modified.
- 75. No compliance certification is made by this report.